

KPMG Advisory Task 3 – Resources

Note: The following resources are **fictional** and were created for this simulation.

Resource 1: Healthcare trends blog

Date: May, 2015

Excerpt: Patient wait times are a persistent challenge for healthcare facilities, affecting both patient satisfaction and operational efficiency. This blog highlights practical, straightforward solutions that healthcare leaders can implement to see immediate improvements:

1. **Expand front desk capacity:** Increasing the number of check-in windows and staffing during busy hours ensures faster registration and reduces long queues at the front desk.
2. **Introduce self-check-in kiosks:** Self-check-in kiosks are a simple way to speed up registration while freeing up administrative staff for other tasks.
3. **Hire additional providers:** Bringing on extra physicians, nurse practitioners, or clinical staff during peak hours allows facilities to move patients through consultations more efficiently.
4. **Enhance the patient experience:** Adding amenities such as complimentary Wi-Fi, refreshments, or comfortable seating can reduce frustration and create a more positive impression during wait times.

These strategies are designed to deliver immediate, tangible improvements in patient flow and satisfaction without requiring major structural changes. Facilities that have implemented these steps report smoother check-in processes, shorter waits for provider consultations, and happier patients.

While each healthcare facility is unique, these tried-and-true solutions offer a practical framework for improving the waiting room experience and operational flow.

Resource 2: Vendor solutions whitepaper

Date: October 2022

Excerpt: Long wait times are one of the most persistent challenges facing outpatient clinics and urgent care facilities. This comprehensive report analyzes the impact of implementing automated workflow management systems on patient throughput and operational efficiency. Drawing on data from 5 outpatient facilities that adopted the **Automated Workflow Platform**.

The report presents the following key findings:

1. **Significant reductions in wait times:** Facilities using the platform reported a 32% decrease in total patient wait times, with registration time reduced by an average of 12 minutes and provider consultation delays cut by 19%.
2. **Improved staff productivity:** By automating patient check-in, queue management, and communication alerts, healthcare staff saved up to 2 hours per day on administrative tasks.
3. **Enhanced patient satisfaction:** Surveys conducted post-implementation showed an 88% satisfaction rate among patients, who cited reduced wait times and improved communication as key drivers of their positive experiences.

The report highlights the unique features of the platform, including real-time patient tracking, automated notifications, and dynamic scheduling tools that enable facilities to respond quickly to patient volume fluctuations.

While the findings suggest a clear link between workflow automation and operational efficiency, it is important to note that results may vary based on facility size, staff adoption rates, and patient volume.

Resource 3: Leading consulting company case study

Date: April 2024

A leading consulting company helped a national urgent care provider standardize operations, streamline workflows, and reduce patient wait times using AI-driven solutions.

Client

National urgent care provider

Sector

Healthcare

Project

Operational efficiency and system integration

Client challenge

After acquiring multiple urgent care facilities, a national provider faced operational inefficiencies caused by inconsistent processes and misaligned staff scheduling. Each facility operated independently, leading to workflow bottlenecks and varying levels of patient care. Long patient wait times and frustrated staff were symptoms of the larger integration challenges.

Approach

The consulting company conducted a detailed assessment of operations. Using GenAI and data-driven tools, we identified opportunities for improvement across **people, process, data management and technology**.

- **AI-powered patient intake:** Company implemented AI tools to automate patient registration, prioritize cases based on urgency, and reduce manual administrative burdens.
- **Predictive staff scheduling:** Predictive analytics optimized staff schedules to align resources with peak patient demand, addressing mismatches in staffing levels that contributed to delays.

Change management support ensured that facility staff received the necessary training to adopt the new systems and processes.

Benefits to client

The integration of AI-driven solutions and process improvements resulted in measurable efficiency gains:

- **15% reduction** in patient wait times across participating facilities.
- **12% improvement** in staff utilization through optimized scheduling.

By leveraging AI and data-driven strategies, the client achieved a more efficient, integrated system that enhanced both patient care and staff productivity.

Resource 4: Non-profit research report

Date: November 2023

Overview: This report explores the impact of workflow redesign, staggered scheduling, and staff training on operating room (OR) efficiency in inpatient hospital settings, highlighting key results and implementation challenges.

In many hospital inpatient settings, delays in OR turnover and inefficient staff scheduling remain persistent challenges to operational efficiency. This report examines the outcomes of process optimization strategies implemented across six large hospitals participating in a surgical efficiency initiative.

Key interventions included:

- Staggered surgical schedules to evenly distribute peak workloads.
- Pre-operative staff training to improve readiness and time management.
- Workflow redesign to streamline equipment turnover and communication among surgical teams.

The initiative led to a 25% reduction in average OR turnover times (from 48 minutes to 36 minutes) and a 16% improvement in staff utilization rates. Interviews with healthcare providers revealed enhanced team coordination and preparedness, contributing to greater efficiency.

However, challenges such as staff resistance to new schedules and resource constraints during high-demand periods were also reported.

Resource 5: Peer-reviewed journal article

Publication date: February 2025

Volume: 12

Issue: 2

Abstract

Prolonged wait times in outpatient urgent care facilities pose significant challenges to care delivery and patient satisfaction in the U.S. healthcare system. This study evaluates the impact of three low-tech interventions—block appointment scheduling, workflow streamlining, and targeted staff retraining—on reducing wait times and improving operational efficiency.

Methods: The study utilized a mixed-method explanatory sequential design over two weeks in February 2024. Quantitative data were collected from 250 patient surveys and analyzed using descriptive and multivariable regression methods. Qualitative data from 12 in-depth interviews (six healthcare providers and six patients) were thematically analyzed to capture insights into patient flow and operational barriers.

Results: The interventions achieved a 28% reduction in median wait times, decreasing from 3 hours and 45 minutes to 2 hours and 40 minutes.

Specific time reductions included:

- **Registration:** Reduced by six minutes.
- **Triage:** Reduced by eight minutes.
- **Provider consultation:** Reduced by 12 minutes.

Regression analysis showed that staff retraining significantly improved triage efficiency (AOR = 2.10, 95% CI: 1.20–3.50, $p = 0.025$). Interviews with patients and providers highlighted improved patient flow, clearer communication, and increased patient satisfaction, though challenges persisted, including inconsistent adherence to new protocols.

Conclusion: The combination of block scheduling, workflow improvements, and staff retraining proved to be an effective and practical solution for reducing urgent care wait times. Sustaining these results requires continued staff training, adherence monitoring, and investments in physical infrastructure and resource allocation.